

Question:

A mental health volunteer is interviewing locals as part of a community outreach program. A 46-year-old man discloses that he has felt sad for as long as he can remember. He feels as though his life is cursed and if something terrible can happen to him, it usually will. He has difficulty making decisions and feels hopeless. He also feels that he has had worsening suicidal ideations, guilt from past problems, energy, and concentration over the past 2 weeks. He is otherwise getting enough sleep and able to hold a job. Which of the following statement best describes this patient's condition?

Answer Choices:

- A. The patient is at risk for double depression.
- B. The patient may have symptoms of mania or psychosis.
- C. The patient is likely to show anhedonia.
- D. The patient likely has paranoid personality disorder.

Reasoning + Answer:

<think>

Finding Reasoning Paths :

- 1 . **Sadder mood â Prolonged sad mood â Major depressive disorder â Paranoia and delusions . **
- 2 . **Suicidal ideation â Negative symptoms of major depressive disorder â Paranoia . **
- 3 . **Poverty and high-risk lifestyle â Behavioral abnormality â Major depressive disorder â Paranoia . **
- 4 . **Energy and concentration â Sleep normalization and normal appetite â Persistent depressive disorder â Paranoia . **

Reasoning Process :

- 1 . **Step 1 : Identify the core symptoms . **
- The patient reports a **prolonged sad mood**, **hopelessness**, **poverty**, **worsening suicidal ideations**, and **deterioration in energy and concentration** . These symptoms align with a **major depressive disorder (MDD)** , particularly with **severe symptoms** .
- 2 . **Step 2 : Explore the connection between MDD and paranoia . **
- In severe or prolonged MDD , individuals may experience **negative symptoms** , such as **paranoia** or **delusions of guilt** , as a manifestation of the disorder . This is described as **double depression** , where individuals experience severe depressive symptoms alongside paranoia or delusions .
- 3 . **Step 3 : Evaluate the patient's risk factors . **
- The patient has **poverty** and a **high-risk lifestyle** , which are known to exacerbate depressive symptoms and may increase the likelihood of paranoid delusions . These factors could contribute to the development of **double depression** .
- 4 . **Step 4 : Rule out other possibilities . **
- The patient reports **normal sleep** and **normal energy** during the week , which makes **acute mania** or **schizophrenia** less likely . Similarly , **paranoid schizophrenia** or **tertiary Schizophrenia** is unlikely given the absence of psychotic features and the context of a depressive disorder .
- 5 . **Step 5 : Assess the likelihood of persistent depressive disorder . **
- Persistent depressive disorder (dysthymia) is characterized by a chronically depressed mood for at least 2 years . However , the patientâs symptoms are acute , and there is no history of a chronic depressive state . Therefore , this is less likely .

Conclusion :

The patient is at high risk for **double depression** , which is characterized by **prolonged major depressive disorder with paranoid delusions or delusions of guilt** . This condition explains the combination of depressive symptoms and paranoid ideations observed in this patient .

</think>

<answer>

The patient is at risk for double depression .

</answer>

